BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Blood and organ safety	Full blood count, kidney and liver tests and electrocardiogram (ECG). Heart-pumping assessment with an echo or multigated acquisition (heart pumping) scan (MUGA) scan if over 65 or clinically indicated; doxorubicin requires left ventricular ejection fraction (heart pumping measurement) (LVEF) above 50%. Hearing assessment and measured kidney clearance when indicated.	Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.	Keep a current dose and symptom record.	Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00615 v2: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests and ECG. Heart-pumping assessment with an echo or MUGA scan if over 65 or clinically indicated; doxorubicin requires LVEF above 50%. Hearing assessment and measured kidney clearance when indicated.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

CAP: advanced salivary gland cancer: treatment and checks

Advanced salivary gland cancer, after specialist review of histology, treatment choices and goals.

Doxorubicin 50 mg/m², cisplatin 50 mg/m² and cyclophosphamide 500 mg/m² on Day 1, repeated every 21 days for up to 6 cycles, stopping earlier for progression or unacceptable side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests and ECG. Heart-pumping assessment with an echo or MUGA scan if over 65 or clinically indicated; doxorubicin requires LVEF above 50%. Hearing assessment and measured kidney clearance when indicated.	Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.	Keep a current dose and symptom record.	Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

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Treatment and review

Day 1 every 21 days; up to 6 cycles

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m^2.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.50